NUPCO

A National Transformation Model: Value-Based Procurement for Wound Care

12-Month Pilot Programme — Diabetic Foot Ulcers

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Executive Summary

Shifting wound care procurement from unit cost to patient outcomes — a scalable national blueprint

- Outcome-Driven Procurement — pay for healing, not products
- 12-Month DFU Pilot with live KPI monitoring across participating hospitals
- Multidisciplinary execution combining clinical, data, and procurement expertise
- Scalable National Blueprint replicable across chronic disease categories
- Full alignment with Saudi Vision 2030 healthcare transformation goals

The Challenge

The Challenge

High DFU Prevalence

18.3% adult diabetes prevalence in KSA; 34% lifetime DFU risk for diabetics — one of the highest burdens globally.

Volume-Based Incentives

Current procurement rewards product volume, not outcomes. Suppliers have no incentive to prevent complications or reduce hospital stays.

Severe Complication Rates

31% of DFU cases lead to amputation; 5-year post-amputation mortality is 42%. The human and economic cost is preventable.

No Outcome Measurement Framework

Absence of standardised KPIs across hospitals means there is no visibility into care quality or supplier contribution to outcomes.

By the Numbers

Baseline burden metrics for Diabetic Foot Ulcers in the Kingdom of Saudi Arabia.

18.3%

Adult Diabetes Prevalence

34%

Lifetime DFU Risk

31%

DFU Cases → Amputation

42%

5-Year Post-Amputation Mortality

Our Approach

Our Approach

Sigil proposes a comprehensive Value-Based Procurement framework for Diabetic Foot Care, grounded in an integrated care pathway spanning screening through long-term maintenance. The model links supplier contracts to measurable patient outcomes, creating a shared-risk structure where procurement cost tracks directly with clinical performance.

Outcome-Linked Contracts

Procurement agreements tied to Tier 1 clinical outcomes (healing rate, amputation rate), Tier 2 patient-reported outcomes, and Tier 3 cost-of-care metrics.

Data Infrastructure

Robust real-time dashboards monitoring KPIs across the pilot cohort, enabling monthly performance reviews and rapid course-correction.

Supplier Prequalification

Structured supplier assessment framework measuring impact on pathology (healing efficacy, complication reduction) and on providers (training, supply chain, cost efficiency).

Why Sigil

Proven VBHC methodology

Sigil brings a tested framework for value-based healthcare implementation, adapted for the Saudi procurement and regulatory context.

End-to-end execution

From stakeholder alignment through pilot launch to national blueprint — one partner owns the full delivery chain.

Academic output

Pilot findings are structured for peer-reviewed publication, creating lasting scientific credibility for the NUPCO model.

Project Roadmap

Project Roadmap

Phase 1 — Analysis & Design
3 months

ACTIVITIES
- Stakeholder mapping and alignment workshops
- Current state assessment and patient journey mapping
- KPI framework definition and baseline measurement
- Draft legal and contract framework for VBP

DELIVERABLES
- Stakeholder map
- Clinical baseline report
- KPI dictionary
- Draft VBP contract

Phase 2 — Setup & Training
3 months

ACTIVITIES
- Contract finalisation and signing
- IT and data integration setup
- Clinical staff training on new care pathways
- Supply chain logistics and dry-run simulation

DELIVERABLES
- Signed VBP contract
- Live data dashboard
- Staff certification records
- Readiness audit report

Phase 3 — Pilot Implementation
3 months

ACTIVITIES
- Go-live of VBP pilot across participating hospitals
- Real-time KPI monitoring
- Patient enrolment and tracking
- Monthly performance reviews and clinical huddles

DELIVERABLES
- Monthly KPI reports
- Patient registry
- Issue log and resolution tracker
- Interim financial report

Phase 4 — Evaluation & Scale
3 months

ACTIVITIES
- Comprehensive outcome and ROI analysis
- Total cost of care calculation
- Model refinement into national VBP blueprint
- Academic research and case study production

DELIVERABLES
- Final impact report
- National VBP blueprint
- Scaling roadmap for other chronic diseases
- Peer-reviewed manuscript submission

Investment

Investment

Phase 1 — Analysis & Design • 3 months

SERVICE	QTY	RATE USD	TOTAL USD
Stakeholder mapping and alignment workshops	1 project	$45,000	$45,000
KPI framework and baseline measurement	1 project	$30,000	$30,000
Legal and contract framework design	1 project	$25,000	$25,000
Phase Total			**$100,000**

Phase 2 — Setup & Training • 3 months

SERVICE	QTY	RATE USD	TOTAL USD
IT and data infrastructure setup	1 project	$60,000	$60,000
Clinical training programme	1 project	$20,000	$20,000
Phase Total			**$80,000**

Phase 3 — Pilot Implementation • 3 months

SERVICE	QTY	RATE USD	TOTAL USD
Programme management and monitoring	3 months	$20,000	$60,000
Phase Total			**$60,000**

Phase 4 — Evaluation & Scale • 3 months

SERVICE	QTY	RATE USD	TOTAL USD
Outcomes analysis and blueprint development	1 project	$50,000	$50,000
Scientific publication and dissemination	1 project	$20,000	$20,000
Phase Total			**$70,000**

TOTAL ENGAGEMENT **USD $310,000**

* Prices are exclusive of applicable VAT.
* Payment terms to be aligned with NUPCO procurement cycles.
* Pass-through costs (travel, venue, expert honoraria) are billed at cost.

Our Team

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Tiago Taveira-Gomes, MD PhD
Project Strategic Lead

- Real World Evidence & Implementation Science
- Data Science & Machine Learning in Healthcare
- AI in Healthcare / Digital Health
- VBHC Programme Design

João Gancho Figueiredo, MD
Senior Clinical Lead

- Clinical Strategy and Evidence Generation
- Healthcare Interoperability
- Health Plan Consultancy
- Clinical-to-Technical Translation

Carlos Sousa, MsC
IT Governance Director

- Healthcare Information Systems Implementation
- Digital Health PMO and Governance
- Interoperability and Cybersecurity in Health

Marco Lima, MsC
Data Engineering Specialist

- SQL Development and Database Management
- ETL Pipeline Development and Interoperability
- Healthcare Application Management
- Clinical Information Systems Integration

Next Steps

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We propose the following immediate next steps to begin the engagement:

1

Proposal Review Call

30-minute session with the NUPCO team to walk through the proposal, clarify scope, and align on pilot hospital selection.

2

Formal Partnership Agreement

Drafting and signing of the framework agreement, including the risk-sharing contract structure and data processing addendum.

3

Phase 1 Kick-off

Stakeholder alignment workshop to launch Phase 1 and begin the current state assessment across the pilot hospitals.

Procurement that pays for health.

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